



Fiona Stanley Fremantle Hospitals Group

OFFICIAL

Guideline

Urgent Mental Health Assessment for Persons 65 Years and Over

Reference #: FSFH-MENH-GUI-0007

Scope

Site	Service/Department/Unit	Disciplines
Fremantle Hospital	Mental Health Service	Medical, Nursing, Allied Health

1. Introduction

Fremantle Older Adult Mental Health Service (FOAMHS) is a community mental health service, operating Monday to Friday 08.00 to 16.00. FOAMHS does not provide an emergency or after-hours service. The purpose of this guideline is to clarify the roles and responsibilities of mental health clinicians in ensuring timely and safe assessment and treatment of any persons 65 years+ who self-present to Fremantle Hospital with a suspected mental illness.

2. Terminology

Urgent Assessment	Refers to presentations having a level of risk indicating that an immediate response is required.
Older Adult	As per the Australian Institute of Health and Welfare, an older adult is defined as an individual aged 65 years and over, whilst for older Aboriginal people, the age range 50 years and over is used to reflect the gap in life expectancy between Aboriginal and non-Aboriginal Australians.

3. Guideline

3.1. Referrals

- In hours all written and verbal and electronic referrals are directed to the FOAMHS triage officer.
- Referrals are reviewed at the FOAMHS daily intake meeting to determine urgency, allocate a care coordinator and schedule an appointment in accordance with the SMHS Care Coordination in Mental Health policy.
- If a person over the age of 65 years self-presents to the Mental Health Crisis Centre (MHCC) within business hours an initial screening assessment, including Risk Assessment and Management Plan (RAMP) will be completed by the Triage clinician.
- Depending on this assessment, the individual is referred on to the most appropriate treating agency e.g. FOAMHS or nearest Emergency Department.

3.2. Mental Health Crisis Centre (MHCC)

- The following patients presenting to MHCC during business hours (08:00 – 22:00 hrs) will be seen by the MHCC:
 - Aged 65 years and over
 - Currently active with FOAMHS
 - Previously active with FOAMHS, currently discharged
 - 65 years and over, active with adult service within last two years
 - 65 years first presentation

3.3. FOAMHS

- During business hours:
 - The FOAMHS Triage officer or care coordinators may provide telephone assistance to MHCC providing additional information, advice, and recommendations regarding any active clients depending on the availability of FOAMHS clinicians.
 - An active client presenting in person to FOAMHS in crisis may be redirected to MHCC for assessment if FOAMHS resourcing is insufficient to provide safe and timely care.

4. Compliance/Performance Monitoring

- The Programme Manager, Older Adults will monitor incidents related to referrals / process against this guideline.
- Any referrals that do not meet the criteria or presentations outside the documented scope will be referred to the Head of Service for further review.

5.Related Policy Documents

- Department of Health
 - [MP 0155/21 Statewide Standardised Clinical Documentation for Mental Health Services](#)
 - [MP 0074/17 Clinical care of people who may be suicidal](#)
 - CI
- FSFHG
 - Clinical Risk Assessment and Management
 - Older adult mental health referrals
 - Waitlisting for older adult mental health admission procedure

6.Related Standards

- NSQHS Standards:
 - Clinical Governance
 - Comprehensive Care
 - Communicating for safety
- Chief Psychiatrists Standards for Clinical Care:
 - Assessment
 - Risk assessment and management plan

7.Authorisation

EXECUTIVE SPONSOR: Mental Health Service Director					
Version	Date Issued	Compiled/Revised By	Committee/Consumer Group Consulted	Endorsed By	Revision due
1	07/2016	OAMH Program Manager	FH MHS CGC	PLCC	07/2019
2	09/2019	OAMH Program Manager	Service 5 SQR Committee	FSFHG Policy Committee	09/2022
3	09/2022	OAMH Program Manager	Service 5 SQR Committee	Service 5 SQR Committee	09/2025
4	11/2025	CNS – Clinical Improvement	FH Clinical Governance Sub Committee	FH Clinical Governance Sub Committee	12/2028